



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : سماح حسين شبل حسين
المحافظة : القاهرة
الحى / المدينة : دار السلام
رقم البطاقة : 28608150108547 رقم التليفون : 01201166066
العمر (تاريخ الميلاد) : 15-08-1986 المهنة : ربه منزل
العنوان : =
تاريخ تسجيل الإستمارة : 30-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبتات
الاروماتيز [14161]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سماح حسين شبل حسين National ID: 28608150T08547	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

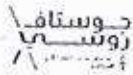
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

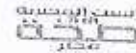
وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



06/16/2026

MDT FORM

Name:	Samah Huseen Shabl Huseen	MRN:	2904
Age:	40 Y	ID:	28608150108547
Menopausal Sta		Phone:	01201166066
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX:

R

L

ER:

PR:

HER2:

KI67:

Sonomammography:Mets Workup:History:

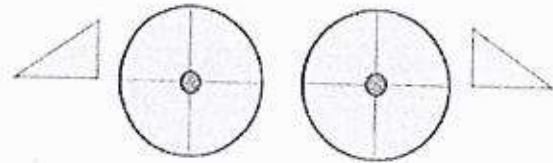
Ct2 N0 , clip inserted rechecked neoadj AC x3 TXT x3 + zoladex for your evaluation regarding surgery

Recommendations:

FOR WIRE local and BCS + SLN

MDT Outcome & Plan

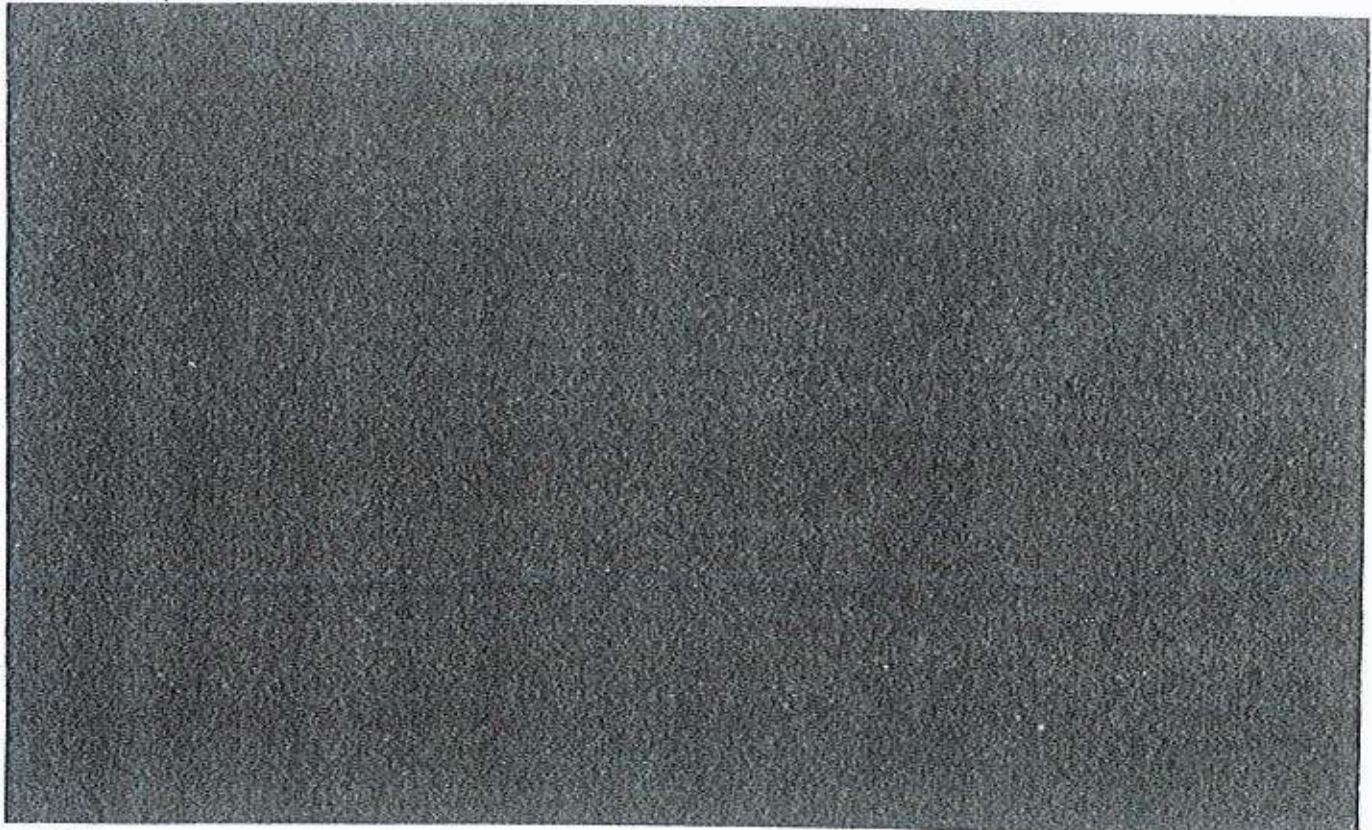
FOR LT WIRE local and BCS + SLN after surgical exam as cesm shows large mass 8cm with overlying skin dimpling



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هبة عبد الحليم	د. هبة عبد الحليم				



LH7158814

٢٨٧ . ٨ . ٨ ٢٦ . ٥١ ٦٧

المستقر - القاهرة
في الرشيد رش القطن مهران
صباح
ثابت محمود عبد المجيد

مركز البحوث الزراعية
القاهرة

LH7158814

787. A. 877. 01 7V

صباح
ثابت محمود عبدالمجيد
في الرشيد ارض القطش مهران
البياتين - القاهرة

مجلس القضاء الاعلى
بغداد - العراق



970

ثابت محمود عبد المجيد

٢٠٠٥ : الرشيد، م. : القحطاني، م. : مهران

المصنفين - القائلون

4. Ref. 8

Allele: LAY-V-NVA

Table 1. *Summary of the results of the regression analysis*

حاصلہ کی تعلیم

2007

Notes

المجلد
العدد
الطبعة

عزت محمد رشاد محمد

المطبخ الآف، سامريية، حش



Name: سماح حسين شبل حسين			
MR. : 2904			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 40-year-old, premenopausal individual with a positive family history and no documented co-morbidities. The clinical history notes a clinical stage T2 N0 disease, placement of a clip, and receipt of neoadjuvant chemotherapy consisting of AC ×3, followed by TXT ×3, together with Zoladex. Biopsy demonstrated invasive ductal carcinoma, grade 2. Hormone-receptor testing was strongly positive with ER 8/8 and PR 8/8, HER2 was 0, and Ki-67 was 50 %. Post-operative sonomammography after left wide local excision (WLE) with skin over the mass and sentinel lymph node biopsy showed residual viable invasive breast carcinoma, not otherwise specified, grade 2 (cellularity 80 %). Margins were free, 0 of 6 lymph nodes were involved, pathology reported a pathologic complete response (PPR), stage T2 N0, and the operative site was marked with intra-operative clips. The multidisciplinary team recommended postoperative radiotherapy (PORT) and prescribed Letrozole plus Zoladex for five years, together with Zometa administered every six months.			
تحريراً في ,			
		التاريخ : 23-09-2026	الوقت :
		الطبيب المعالج :	
يعتمد ,			



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



06/16/2026

MDT FORM

Name:	Sabah Sabet Mahmoud Abdelmeged	MRN:	9747
Age:	39 Y	ID:	28708082605167
Menopausal Sta		Phone:	01104892370
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	NEW

Clinical Data:

BX:

ER: 0 PR: 0 HER2: +3 KI67: 90

Sonomammography:**RIGHT BREAST:**

Heterogenously dense breast parenchyma that may obscure small lesions (ACR C).

outer breast large irregular speculated high density mass lesion reaching the nipple that is seen retracted with marked surrounding dotorion and suspicious pleomorphic microcalcifications within.

complimentary U/S demonstrated outer breast 8-10 o'clock, middle and inner thirds, abutting the overlying skin and the underlying muscle, irregular antiparallel heterogenous speculated solid mass lesion with moderate internal vascularity and microcalcifications measuring 55 x 60 mm sear reaching the nipple.

Another similar smaller lesion is seen at the axillary tail measuring 17 x 17 mm ?? nodel; ?

primary.

Normal skin/nipple/areola complex.

Few (more than three) level 1 & level 2 pathological lymph nodes are noted. No pathologica lateral infra or supra-clavicular lymph nodes.

Mets Workup:

BONE SCAN/ FEW OSSEOUS LESIONS

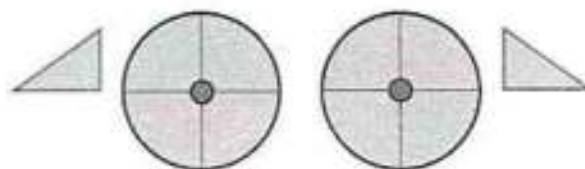
16-4-2026: CT CAP: D9 vertebral body, left iliac bone lytic lesions

History:**Recommendations:****MDT Outcome & Plan**

For Herceptin - Paclitaxel + Zometa / 3m

R

L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient's Name: Sabah Thabet Mahmoud Abdelmegi
Date: 2026-04-16
Referred by: Prof.Dr. CAPwithC
Hospital: Goustave Roussey International Egypt

9787

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

I- CHEST:

- Known case of right breast cancer showed right breast mass lesion measuring 51 x 48 mm showed heterogenous enhancement being inseparable from the right pectoralis major muscle
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Post partial colectomy status with n operative bed masses or collections
- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- D9 vertebral body, left iliac bone (ischial tuberosity and acetabular posterior wall and roof) lytic lesions

Opinion:

- ❖ Known case of right breast cancer showed no hepatic or pulmonary metastatic lesions.

Patient's Name: Sabah Thabet Mahmoud Abdelmegi
Date: 2026-04-16
Referred by: Prof.Dr. CAPwithC
Hospital: Goustave Roussey International Egypt

❖ D9 vertebral body and left iliac bones lytic lesions likely metastatic for bone scan

Dr.Assem Ahmed

Dr.Assem Ahmed

Patient Name :	الأسنانة / صباح ثابت محمود	Request No. :	13005432
Patient No. :	28708082605167	Request Date :	12/03/2026
Path No. :	695/26	Verification Date :	18/03/2026
Sex :	Female	Financial Category :	جوستاف روسي - مبادره
Age :	38 Y	Department :	Out Patient
Referreing Doc :		Registered by :	.marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 12/3/2026, at 12:45 pm, Cold ischemia time is 20 seconds.

Gross Examination:

- Three grey white tissue cores, the longest measured 1.3 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angioinvasion: Absent.
- * Necrosis: Widely necrotized.

Diagnosis:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3.

For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghonaim.

Verified by :

Printing Date : 02/04/2026 11:39:33

Page 1 of 1

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / صباح ثابت محمود	Request No. : 13005432
Patient No. : 28708082605167	Request Date : 12/03/2026
Path No. : 695/26	Verification Date : 18/03/2026
Sex : Female	Financial Category : جوماتف روسي - مبادره
Age : 38 Y	Department : Out Patient
Referreing Doc. :	IHC No. :
	Registered by : .marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass (BIRADS 5), diagnosed invasive duct carcinoma grade 3.
Nature of Specimen: Tru-cut biopsy, collected on 12/3/2026, at 12:45 pm, Cold ischemia time is 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8)
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 90%.

Conclusion:

Right breast mass, Tru-cut biopsy:

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67%: 90%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2,

Verified by :



Patient Name الأستاذ / صباح ثابت محمود
Patient No. 28708082605167
Path No. 695/26
Sex Female
Age 38 Y
Referring Doc.

Request No. 13005432
Request Date 12/03/2026
Verification Date 18/03/2026
Financial Category جوسفاروسي - مبادره
Department Out Patient

Registered by .marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

Intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

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Printing Date : 02/04/2026 11:39:34

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name Sabah Thabet Mahmoud

Date : 19 May 2026

DUPLEX EXAMINATION OF THE VENOUS SYSTEM OF THE RIGHT UPPER LIMB REVEALED: -

Deep Venous System: -

- *The axillary, rest of brachial veins down to radioulnar veins all are patent, totally compressible with normal color filling & preserved respiratory variation.*

Superficial Venous System: -

- *Patent cephalic & basilic vein.*
- *Diffuse limb edema as skin inflammatory changes*

OPINION: -

*No DVT nor left UL superficial thrombophlebitis.
Diffuse subcutaneous edema*

*Best Regards,
Dr/Abubakr Hejazi MSc FEBR*

Name : SABAH THABET MAHMOUD

Date : 17-02-2026 03:30

Branch : Portable

Medical No. : GRI-170226030408PM

Ref.Physician Prof :

Ins.Company :

**BILATERAL DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH
RESOLUTION BREAST SONOGRAPHY REVEALED**

CLINICAL SUMMARY:

Left breast palpable lump

FINDINGS:

LEFT BREAST:

- Heterogenously dense breast parenchyma that may obscure small lesions (ACR C).
- UOQ, middle third two circumscribed oval fat containing mass lesions, no masses, asymmetry, distortion or suspicious micro-calcifications.
- complimentary U/S demonstrated two 2 o'clock, middle third indeterminate intra-mammary lymph nodes in terms of having hypoechoic lobulated cortices yet not exceeding 3 mm in thickness; measuring 17 x 8 mm & 10 x 6 mm.
- Non specific axillary nodes are noted.

RIGHT BREAST:

- Heterogenously dense breast parenchyma that may obscure small lesions (ACR C).
- outer breast large irregular speculated high density mass lesion reaching the nipple that is seen retracted with marked surrounding distortion and suspicious pleomorphic microcalcifications within.
- complimentary U/S demonstrated outer breast 8-10 o'clock, middle and inner thirds, abutting the overlying skin and the underlying muscle, irregular antiparallel heterogenous speculated solid mass lesion with moderate internal vascularity and microcalcifications measuring 55 x 60 mm seen reaching the nipple.
- Another similar smaller lesion is seen at the axillary tail measuring 17 x 17 mm ?? nodal; ?? primary.
- Normal skin/nipple/areola complex.
- Few (more than three) level 1 & level 2 pathological lymph nodes are noted. No pathological lateral infra or supra-clavicular lymph nodes.

CONCLUSION:

Left breast (BIRADS 3): UOQ two indeterminate intra mammary lymph nodes, probably reactive, yet for US guided Tru-cut biopsy and histopathological correlation.

Right breast (BIRADS 5): locally advanced highly suspicious process with pathological axillary level 1 & 2 lymph nodes, for US guided Tru-cut biopsy & histopathological correlation.

Best regards,
Dr. Amal Hegazi, MSc

Patient's Name: Sabah Thabet Mahmoud Abdelmegi
Date: 2026-04-16
Referred by: Prof.Dr. CAPwithC
Hospital: Goustave Roussey International Egypt

❖ D9 vertebral body and left iliac bones lytic lesions likely
metastatic for bone scan

Dr. Asser Ahmed

Dr. Asser Ahmed

Patient Name :	الأستاذ / صباح ثابت محمود	Request No. :	13005432
Patient No. :	28708082605167	Request Date :	12/03/2026
Path No. :	695/26	Verification Date :	18/03/2026
Sex :	Female	Financial Category :	جوستاف روسي - ميلاده
Age :	38 Y	Department :	Out Patient
Referreing Doc. :		IHC No. :	
		Registered by :	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass (BIRADS 5), diagnosed invasive duct carcinoma grade 3.
Nature of Specimen: Tru-cut biopsy, collected on 12/3/2026, at 12:45 pm, Cold ischemia time is 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

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- * IPT (All controls showed appropriate reactivity):
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- Ki67%: 90%.

Conclusion:

Right breast mass, Tru-cut biopsy:

IPT:

- ER: Negative (score 0/8).
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- Her2: Positive (score 3+).
- Ki67%: 90%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2,

Verified by :



Patient Name : الأستاذ / صباح ثابت محمود	Request No. : 13005432
Patient No. : 28708082605167	Request Date : 12/03/2026
Path No. : 695/26	Verification Date : 18/03/2026
Sex : Female	Financial Category : جوسفاروسي - مبادره
Age : 38 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

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Gross Examination:

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Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angioinvasion: Absent.
- * Necrosis: Widely necrotized.

Diagnosis:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3.

For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghonaim.

Verified by :



Patient Name الأستاذ / صباح ثابت محمود
Patient No. 28708082605167
Path No. 695/26
Sex Female
Age 38 Y
Referring Doc.

Request No. 13005432
Request Date 12/03/2026
Verification Date 18/03/2026
Financial Category جوستاف روسي - مبادرة
Department Out Patient
Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

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Printing Date : 02/04/2026 11:39:34

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: Sabah Thabet Mahmoud

ID: 5187878

Date: 23/05/2026

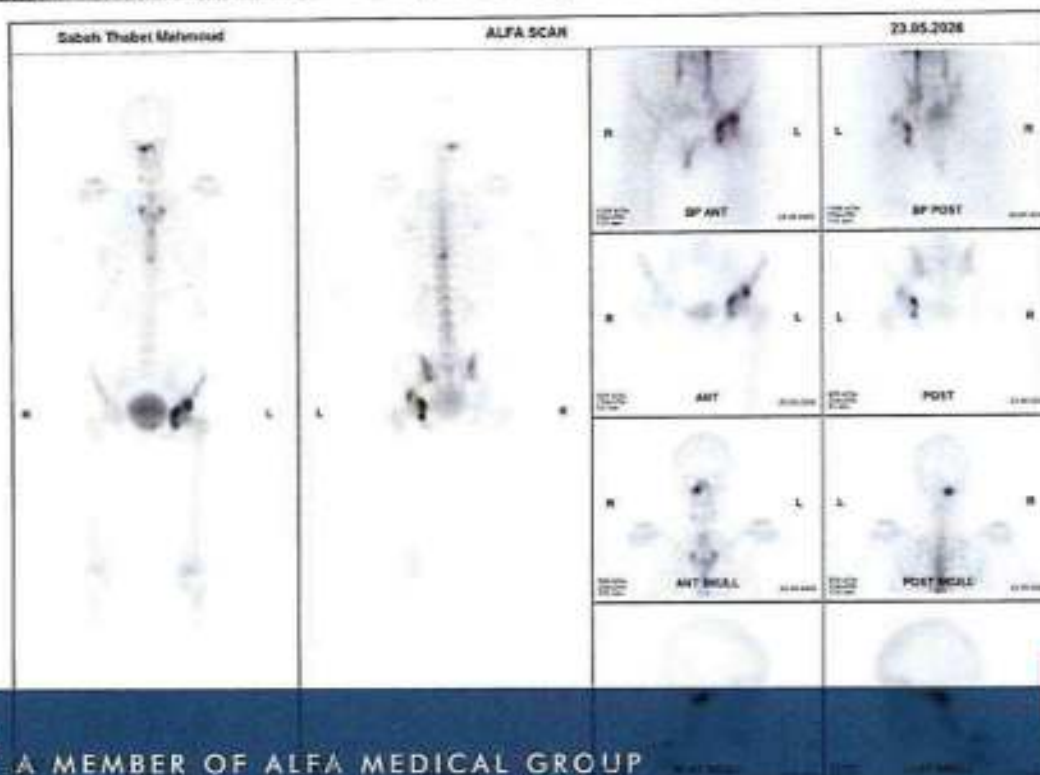
^{99m}Tc-MDP Triple Phase Bone Scan

Clinical data:

complaint	Left hip pain started since 7/2025. Recently diagnosed right breast cancer.
Labs	Not available.
Skeletal trauma/ surgeries	Recurrent trauma due to fall.
Previous imaging	Not available.
Previous bone scan	No.

Technique:

Injected dose	740 MBq
Images	Dynamic, blood pool and whole body delayed images of SPECT on pelvis and spot views of pelvis and skull.
Complementary images	CT skull, dorsal vertebrae and pelvis.



Findings:

- Increased osseous tracer uptake is seen involving the left acetabulum with corresponding hyperemia in the early dynamic and blood pool images. Complementary CT images show corresponding lytic lesion.
- Increased osseous tracer uptake is seen involving the right part of the skull base and DV9 with corresponding lytic lesions in the complementary CT images.



Impression:

- Few osseous lesions as described, likely metastatic.

Dr. Sherif El Refaei, MD, FEBNM

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